



Fiona Stanley Fremantle Hospitals Group

Policy and procedure

This policy is currently under review. It has been assessed by content experts and considered fit for purposes.

Physical health care: Community Mental Health Consumers

Reference #: FSFH-MENH-POL-0001

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Service	Medical and Nursing

1. Introduction

It is recognised that the physical health of people with a mental illness is poor and that poor physical health is associated with impaired mental health. People with severe mental illness are known to have high rates of mortality, reduced life expectancy and decreased healthcare access.

Mental health services are well placed to support improvement in a mental health consumer's physical health and facilitating or advocating for the provision of this care is the responsibility of the Mental Health Service. This policy and procedure defines the expected standards for monitoring and supporting the physical health care of all active Community Mental Health Consumers to ensure identification and active management of current and ongoing needs.

2. Terminology

Relevant history	For the purposes of the policy the core components of initial assessment include: • current prescribed medications • drug and alcohol use assessment including smoking
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	• presence of new physical health problems or symptoms of concern • known presence of diabetes, high blood pressure, high cholesterol, respiratory illness, other illness • relevant family history When conducted as part of a review should also include: • diet • physical activity • current consumer concerns re health issues • preventative health care
Physical examination	For the purposes of this policy the core components include: • observations – blood pressure (BP); pulse and respiratory rate; temperature • weight and waist circumference or waist to hip ratio • height if not already recorded • examination of respiratory, cardiovascular, gastrointestinal and neurological systems

3. Policy

- Physical health care will be delivered in a respectful, non-judgemental and culturally sensitive way with consideration for the consumer's preferences, gender, ethnicity, health literacy and age.
- Interpreter services will be used to facilitate treatment planning processes for consumers from culturally and linguistically diverse backgrounds.
- Clinicians will consult with specialist Aboriginal Health services to ensure their approach to providing physical health care for Aboriginal consumers is consistent with the needs of the local Aboriginal community.
- All community mental health consumers will have a physical healthcare assessment as required, or at minimum yearly.
- All consumers on antipsychotics or medications that contribute to metabolic syndrome will have metabolic monitoring as outlined in the Early Intervention Framework. Refer to [Appendix 1](#) and [Appendix 2](#)
- Physical health and metabolic screening is to be commenced from the consumer's first contact with the Mental Health Service.
- Physical assessment of consumers active with the community mental health service will include a relevant history and physical examination:

- Relevant history and physical examination will include the core components outlined in Section 2.
- All admissions to a community treatment team will have the physical examination within one month of the initial assessment.
- Clinicians completing the assessment will use iSOFT Clinical Manager to request investigations and monitor results.
- The Mental Health Metabolic Monitoring Form will be used as part of ongoing metabolic health monitoring
- All abnormal findings will be actioned or referred to the appropriate health care provider for further investigations

If a physical examination cannot be performed, the reasons will be clearly documented and followed up by the treating team at reasonable intervals until the person is examined.

4. Procedure

4.1. Treating team responsibilities

- Educate the consumer regarding:
 - The importance of having a regular General Practitioner (GP)
 - The need for regular physical examination (yearly at a minimum)
- Where consent allows, contact the consumer's GP in the first instance to discuss previous physical health screening and subsequent investigations.
 - File a copy of previous physical examination conducted by the GP in the consumer's medical record.
- Where the consumer is not engaged with a GP or the GP cannot be contacted conduct a relevant history and physical examination:
 - on activation,
 - on admission to an inpatient Mental Health ward and
 - on an annual basis unless an earlier examination is indicated.
- Consider referral to appropriate allied health e.g. Exercise Physiology, Dietetics, Wellness Clinic

Physical examinations conducted by the mental health service will be documented on the Statewide Standardised Clinical Documentation Mental Health Physical Examination Form and a copy scanned in the patient's medical record.

- Physical investigations will be determined by the consumer's medical history, medications and results from the physical examination
 - Physical investigations for consumers on antipsychotics or medications that contribute to metabolic syndrome will be as per the Early Intervention Framework.
- Involve the consumer in physical care and treatment planning, consider the need to support the consumer to attend scheduled appointments by organising transport.
- At the consumer's three-monthly clinical reviews:
 - Document the consumer's blood pressure, weight and waist circumference
 - update the results documented in the outpatient notes.
 - include the date of the most recent physical examination.
 - update the agreed plan for the consumer's ongoing physical health care on their Psychiatric Services Information System (PSOLIS) Treatment Support and Discharge Plan.

The Consumer will be recognised as part of the care team and encouraged to participate in their physical health care.

- Where the consumer gives verbal permission, the family and/or primary carer will be kept informed regarding the consumer's physical health status and involved in physical care and treatment planning.
- The consumer's GP will be regularly provided with information regarding the consumer's mental and physical health and agreed management plan.
- Wherever possible link the consumer with appropriate primary care and community-based service providers to support their physical health and wellbeing.

4.2. Health promotion

Minimum standards for screening and health promotion interventions for Mental Health Consumers are outlined in [Appendix 3](#)

- Written information will be made readily available to consumers and carers on general physical health issues.
- Access to health promotion, screening and preventative activities will be made available to the consumer such as exercise, relaxation, smoking cessation, diet and weight control.

- Practitioners prescribing medications will comprehensively discuss compliance and discontinuation issues, potential adverse effects and other relevant information with the consumer.
- Consumers will be provided with written information on the impact of medications on their physical health using Choice and Medicine® leaflets from the WA Health Department of Health for psychotropic medications (<https://www.choiceandmedication.org/wadoh/>) and Consumer Medicine Information approved by the Therapeutic Goods Administration for non-psychotropic medications.
- All health promotion and information provided to the consumer will be recorded in the medical record.

4.3. Discharge

- All inpatient discharge summaries will include the following details:
 - relevant results
 - physical examination findings
 - interventions and outcomes
 - physical health issues at discharge
 - follow up arrangements - confirmed
 - follow up arrangements - requiring further follow-up

5. Compliance/Performance Monitoring

The Head of Service, Programme Managers and Nurse Director will monitor compliance with this policy and procedure via annual audits evaluating documentation of the patient's physical health care documented on the consumers Treatment Support and Discharge Plan.

6. Related Policy Documents

WA Health:

- Mental Health Consumer Medication Information Policy
- State-wide Standardised Clinical Documentation for Mental Health Services
- 'Triage to Discharge' Mental Health Framework for State-wide Standardised Clinical Documentation

7. Related Standards

NSQHS Standards:

- Comprehensive care
- Communicating for safety

NSMHS

- Promotion and prevention
- Delivery of care

Chief Psychiatrist Standards for Clinical Care:

- Physical health care of mental health consumers

8. Bibliography

1. NSW Department of Health. 2017. Physical Health within Mental Health services policy
2. NSW Department of Health. 2017. Physical Health Care of Mental Health Consumers guidelines
3. The University of WA. 2010. Clinical Guidelines for the Physical Care of Mental Health Consumers
4. The Royal Australian College of General Practitioners. Guidelines for preventive activities in general practice. 9th ed. East Melbourne, Vic: RACGP, 2016.

9. Authorisation

EXECUTIVE SPONSOR: Nurse Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	03/2018	CNS, Clinical Improvement	Clinical Governance Committee	FSFHG Policy Committee	03/2021
2	07/2021	CNS, Clinical Improvement		Service 5 SQR Committee (Endorsed via Single Speciality Process)	07/2025

10. Appendices

10.1. Appendix 1: Early Intervention Framework

Early Intervention Framework

Baseline and ongoing monitoring recommendations for metabolic syndrome in patients taking antipsychotics

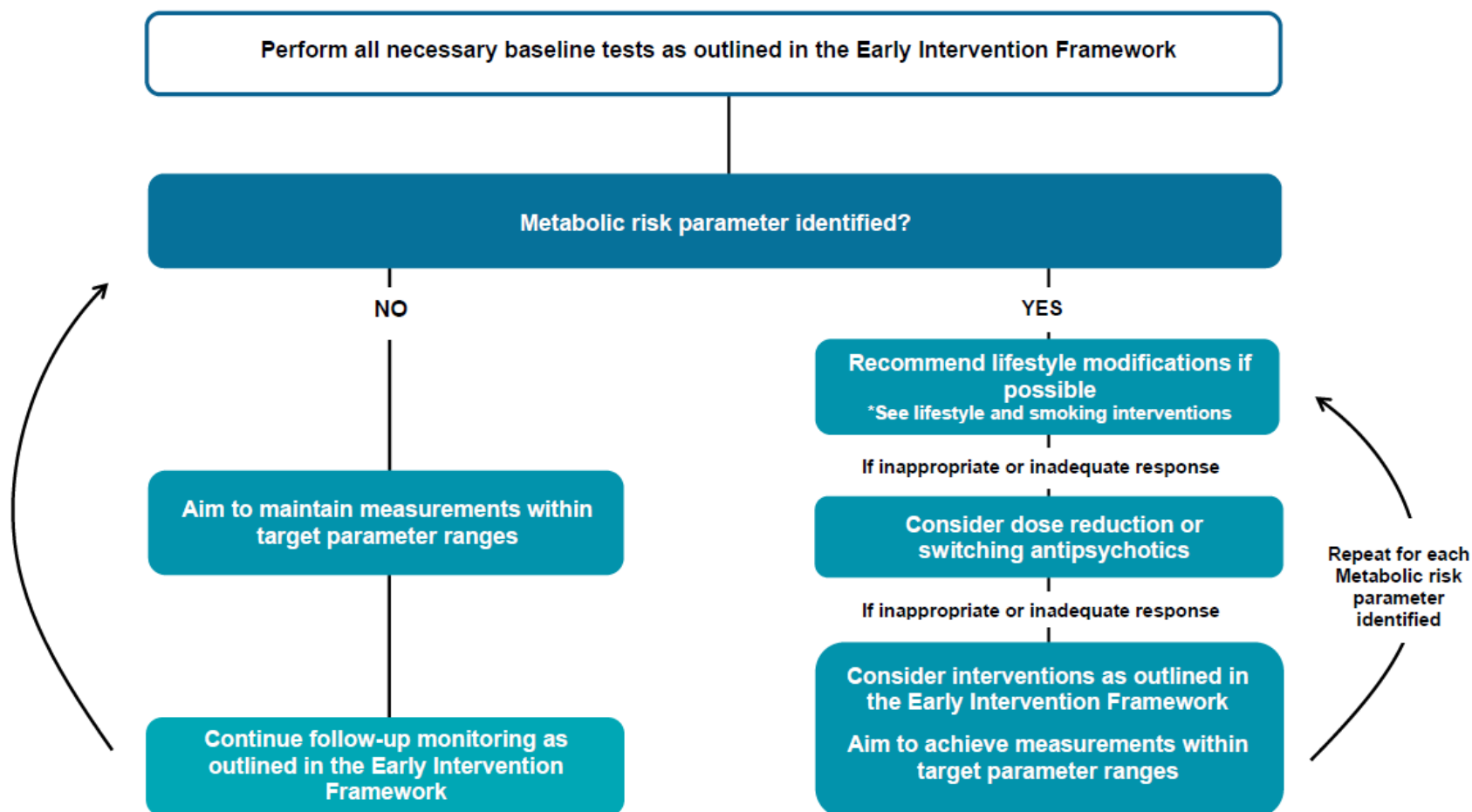
Parameters	Metabolic Risk Parameters	Monitoring Frequency	Interventions	Target Parameters
Family History	Diabetes, CVD, obesity, heart disease, arrhythmia, prolonged QT syndrome, sudden cardiac death	Baseline		
Lifestyle Physical activity Dietary habits	Sedentary lifestyle Excessive alcohol intake Poor nutritional status	Baseline and when clinically indicated	Promote and maintain adherence, implement structured group programs, ≥ 90 minutes of moderate-vigorous (aerobic fitness) activity each week. Reduce alcohol intake and recommend referral to alcohol and drug support programs when required. Consider dietician referral. Cardio-protective Mediterranean diet: plentiful fruits, vegetables, whole-grain cereals, legumes, nuts, moderate fish and poultry, moderate/low red meat. - Reduce sugars, sweetened beverages, fructose, saturated fatty acids and salt - Reduce overall energy intake	Adherence to exercise Increased physical activity levels Improved diet quality Reduced alcohol intake
Smoking Status	Current smoker	Baseline and when clinically indicated	Nicotine replacement therapy (NRT) Smoking cessation program Motivational counselling If ineffective, consider bupropion/varenicline	Smoking cessation
Lipids Triglycerides HDL-C Total Cholesterol	TG: ≥1.7mmol/L HDL-C: Male: ≤1 mmol/L Female: ≤1.3 mmol/L TChol: ≥5.5mmol/L	Baseline, 3 months, then annually* 4-6 weekly during dose titration *Consider increasing for clozapine/olanzapine	Consider statin therapy Alternatives: ezetimibe, fenofibrate	TG: ≤2mmol/L HDL-C: Male: ≥1.03mmol/L Female: ≥1.29mmol/L TChol: ≤4mmol/L
Blood Glucose Fasting BSL HbA1c	Fasting: ≥5.5 mmol/L HbA1c: ≥6-6.4%	HbA1c: Baseline, 3-4 monthly until stable, then 6 monthly Fasting BSL: Baseline, 3 months, then annually	Pre-diabetes: Close assessment and follow-up, referral to endocrinologist. Diabetes: Metformin If contraindicated or ineffective, consider: SU, DPP4i, SGLT2i, GLP-1 agonist	Fasting: Diabetic: 4-8mmol/L Non-diabetic: 4-6mmol/L HbA1c: Diabetic: ≤7% Non-diabetic: ≤6%
Blood Pressure	≥130mmHg systolic and/or ≥85mmHg diastolic	Baseline, 3 months, then 6 monthly	Consider anti-hypertensive ACEi/ARB	Diabetic: ≤140/80mmHg Non-diabetic: ≤130/80mmHg
Obesity Weight, Waist Circumference, BMI	BMI: ≥25kg/m ² Waist: Male: ≥94cm* Female: ≥80cm *90cm South Asian, Chinese or Japanese	Weight/BMI: Baseline, 4, 8 and 12 weeks, then 3 monthly Waist: Baseline then annually	Consider: - Metformin - Adjunct aripiprazole - Adjunct topiramate - GLP-1 agonists	BMI: ≤25kg/m ² Waist: Male: ≤94cm* Female: ≤80cm *90cm South Asian, Chinese or Japanese
Prolactin	>2500mIU/L	Baseline, 6 months, then annually if patient is experiencing symptoms	Rule out prolactinoma Adjunct aripiprazole (2.5-5mg daily) Metformin 1g daily	Male: <340mIU/L Female: <425mIU/L

Professional judgement should be used based on individual patient requirements.
Note: Increased monitoring requirements exist for clozapine

10.2. Appendix 2: Early Intervention Flowchart

Early Intervention Flowchart

Metabolic syndrome in patients taking antipsychotics



10.3. Appendix 3: Minimum standards for screening and health promotion

Activity	Frequency	Notes
Alcohol & other drugs	Yearly and opportunistically	Yearly for low risk groups and more frequently for higher risk populations.
Sexually transmissible disease screening	Yearly	Consumers with significant mental health illness are classified as a higher risk population, suggesting yearly screening. Suggest offering testing for chlamydia, gonorrhoea, hepatitis B, syphilis and HIV.
Absolute cardiovascular disease risk assessment	Two yearly	Calculation of risk requires other screening measures to be performed first. High risk groups may benefit from additional screening.
Dental	Yearly	- Opportunistic examination of the mouth and lips. - At least 12 monthly dental examinations or more frequent depending on dentist recommendations.
Cancer screening tests:		
FOBT	Two yearly	
Mammogram (females only)	Two yearly over the age of fifty	
Cervical cancer - pap smear (females only)	Every five years	
Skin Cancer	Addressed opportunistically	
Prostate cancer (males only)	Addressed opportunistically	